

HOW TO OPEN A BRAIN INJURY TRANSITIONAL LIVING FACILITY IN FLORIDA

A Florida AHCA-licensed Transitional Living Facility (TLF) serving adults with brain and spinal cord injuries
— rehabilitation and community re-entry toward independent living.

A step-by-step startup and operations guide

Licensing (AHCA) • Staff & required training • The rehab program • Funding • Accreditation

Regulatory basis: Chapter 400, Part XI, F.S. (Transitional Living Facilities); Rule 59A-17, F.A.C.; Chapter 408, Part II, F.S. (health care licensing). Funding: Brain and Spinal Cord Injury Program (BSCIP, s. 381.74–381.79, F.S.) and Statewide Medicaid Managed Care Long-Term Care.

How to Use This Guide

This guide walks you through opening and operating a Transitional Living Facility (TLF) in Florida — a home that helps adults with brain and spinal cord injuries rebuild independent-living skills and return to the community. Unlike most states, Florida has a purpose-built license for exactly this: the TLF, licensed by the Agency for Health Care Administration (AHCA) under Chapter 400, Part XI, F.S., and Rule 59A-17, F.A.C. There is one licensing path — get the home licensed by AHCA as a TLF — and a funding picture built around Florida's Brain and Spinal Cord Injury Program (BSCIP), Medicaid managed care, and accident-related insurance rather than a single Medicaid per-diem.

Important — verify before you act

This guide is an educational resource, not legal, financial, tax, or billing advice. Requirements, forms, fees, and rates change. Confirm every step with AHCA (Long Term Care Services Unit) and, for funding, with BSCIP (Florida Department of Health) and the resident's Medicaid plan or insurer before you act. A TLF is transitional and rehabilitation-focused — the goal is community reintegration, not indefinite long-term residence.

Part 1 — What This Business Is

A Florida TLF is a residential facility that assists persons with spinal cord injuries and head (brain) injuries to achieve a higher level of independent functioning in daily living skills. It provides rehabilitative services, community re-entry training, aids for independent living, and counseling. It is its own license category — not an assisted living facility (ALF) and not a nursing home.

- **Population:** adults with a moderate-to-severe brain or spinal cord injury who are medically stable and can reasonably be expected to benefit from transitional rehabilitation and return to a community setting.
- **Purpose:** time-limited, goal-directed rehabilitation and independent-living-skills training — not indefinite custodial care.
- **License:** Transitional Living Facility, issued by AHCA under Chapter 400, Part XI, F.S., and Rule 59A-17, F.A.C.

Part 2 — Who Regulates You

- **AHCA** — Agency for Health Care Administration, Bureau of Health Facility Regulation (Long Term Care Services Unit). Licenses your TLF, sets the Rule 59A-17 standards, and conducts the licensure survey.
- **Florida Department of Health — BSCIP** — the Brain and Spinal Cord Injury Program: case management, funding as payer of last resort, and the Central Registry that receives all new moderate-to-severe TBI/SCI referrals (s. 381.74, F.S.).
- **DCF** — Department of Children and Families: operates the Florida Abuse Hotline for reports of abuse, neglect, or exploitation (Ch. 415, F.S.).

Part 3 — Business and Insurance Setup

- Form your business entity with the Florida Division of Corporations and be in good standing.
- Secure the home (lease or purchase) and confirm local zoning permits a residential care use; Florida's community residential home siting law (Ch. 419, F.S.) may apply.
- Obtain liability and property insurance appropriate to a residential rehabilitation facility.
- Identify all owners and controlling interests — you disclose them on the AHCA application, and staff undergo Level 2 background screening.
- Obtain a National Provider Identifier (NPI) and, if you will bill Medicaid managed care, plan for Medicaid provider enrollment and MCO contracting.

Part 4 — The Home: Physical Plant and Fire Safety

Your building must meet the physical-plant standards of Rule 59A-17.119, F.A.C., and applicable fire-safety requirements. Before your licensure survey, confirm:

- ☐ The building meets the Rule 59A-17.119 physical-plant standards, with accessibility for residents who may have mobility, sensory, or cognitive impairments.
- ☐ Resident rooms, bathrooms, and rehabilitation/therapy and common areas suited to independent-living-skills training.
- ☐ Fire protection — detection, alarms, and systems required for the building — tested and documented (Ch. 69A-40, F.A.C.).
- ☐ A Comprehensive Emergency Management Plan, including Florida's alternate-power / safe-indoor-temperature requirement for power loss, plus hurricane and evacuation planning.
- ☐ Safe hot-water temperatures; hazardous items stored to prevent resident access.
- ☐ Documented physical-plant and safety inspections.

Part 5 — Step 1: Administrator, Staff, and Required Training

- **Administrator:** the facility operates under a qualified administrator responsible for operation, staff, and resident services, with a designated person to act in the administrator's absence.
- **Level 2 background screening:** all staff and contractors with resident contact clear Level 2 screening through the AHCA Clearinghouse (Ch. 435 / s. 408.809, F.S.) before resident contact.
- **Unlicensed direct care training & validation:** unlicensed direct care staff complete the training and validation required by Rule 59A-17.126, F.A.C., before performing covered tasks — including any medication administration allowed under Rule 59A-17.125, F.A.C.
- **Brain-injury competencies:** train staff on cognitive and behavioral effects of brain injury, cueing and compensatory strategies, fall and safety management, and de-escalation.

Part 6 — Step 2: Get Your TLF License from AHCA

1. Register on the AHCA online licensing portal and start the Transitional Living Facility application under Chapter 408, Part II, F.S.
2. Upload the required documents: entity and ownership disclosures, property documents, proof of zoning/occupancy, your Comprehensive Emergency Management Plan, fire-safety documentation, and your Chapter 400 Part XI / 59A-17 policies and procedures.
3. **Pay the license fee.** AHCA sets the fee by rule (Rule 59A-35) on a per-bed formula with a minimum; confirm the current TLF fee on AHCA's licensure-fee schedule — fees are non-refundable, and a late application triggers a \$50/day penalty.
4. Submit at least 60 days before your intended opening date.
5. Pass the AHCA licensure survey; the license is issued once a successful survey is on file. A valid license is required before you serve residents.

Where your manual and forms fit. The application requires complete Chapter 400 Part XI / 59A-17 policies and procedures. Your Policy & Procedure Manual and Forms Packet are built to that requirement — have them ready before you apply.

Part 7 — Step 3: How You Get Paid — Funding the Home

Here is the honest funding picture, because it drives your business model. Florida folded its old standalone brain/spinal-cord-injury Medicaid waiver into managed care on December 1, 2017, so there is no separate brain-

injury waiver — and there is no single published Medicaid per-diem for a TLF. You build funding from these sources:

Source	What it pays / how	Code / rate reality
BSCIP (Dept. of Health)	Dedicated brain/spinal-cord-injury funder; case-managed; contracts for services incl. transitional living. Payer of last resort — other coverage used first.	No fee schedule. Scale: ~\$14.8M served 987 people in FY2023–24. Referral via BSCIP Central Registry.
Medicaid SMMC Long-Term Care	Managed care (the old TBI/SCI waiver folded in on 12/1/2017). MCO pays the facility; requires nursing-facility level of care; room & board not covered.	Assisted-living service bills to the MCO under HCPCS T2031; rate negotiated per MCO contract. Resident keeps ~\$160/mo personal-needs allowance (2026).
Auto (Florida PIP), workers' comp	Common for TBI/SCI because most are accident-related; pays for post-acute rehab and residential care per the policy.	Per the insurance policy / claim; PIP minimum is \$10,000. No single per-diem.
Private pay / vocational rehab	Family private pay; Division of Vocational Rehabilitation for work-reentry services.	Set by your rate schedule / VR authorization.

The one code to know: if a resident is enrolled in Medicaid SMMC Long-Term Care and receives assisted-living services, that service bills to the member's managed care organization under HCPCS T2031 ("Assisted living; waiver, per diem"), at a rate negotiated in your MCO contract. There is no separate published TLF per-diem — confirm any rate in your BSCIP or MCO agreement rather than relying on a fixed number.

Verify each resident's funding before admission

BSCIP is a payer of last resort, SMMC LTC does not cover room and board, and auto/workers'-comp coverage depends on the claim. Confirm each resident's specific coverage — and what it will and will not pay — before you admit.

Part 8 — Staffing, the Rehabilitation Team, and Training

- Deliver services through an interdisciplinary team suited to residents' needs — which may include physical, occupational, and speech-language therapists; neuropsychology or behavioral services; rehabilitation nursing; case management; and trained direct care staff.
- Staff to residents' acuity and independence goals, with awake overnight coverage appropriate to the population.
- Screen (Level 2), train, and validate all staff, and document competency — not just attendance.
- Hold and document regular team meetings on each resident's progress toward community-reintegration goals.

Part 9 — The Rehabilitation Program and Accreditation

A TLF is a rehabilitation program: each resident gets a comprehensive assessment, an individualized rehabilitation plan with measurable community-reintegration goals, independent-living-skills training, neurobehavioral supports, and community re-entry planning from admission — with program-level outcomes tracked over time.

Accreditation — a real market lever. Florida managed-care plans and other payers often require or prefer national rehabilitation accreditation for brain-injury providers. The two recognized frameworks are the Joint Commission and CARF (the Commission on Accreditation of Rehabilitation Facilities, which specializes in brain-injury and residential rehabilitation programs). Building your program to align with these frameworks — without reproducing

their proprietary standards — strengthens both survey readiness and payer contracting. Your Policy & Procedure Manual is structured to support this alignment.

Part 10 — Timeline, Costs, and Getting Your First Residents

Rough sequence

Form your entity and secure/prepare the home → hire and train your administrator and staff (Level 2 screening; 59A-17.126 validation) → complete the AHCA online application with policies, documents, and fee → submit 60+ days ahead → pass the licensure survey → receive your TLF license → line up funding (BSCIP, Medicaid MCO contracts, insurance) and begin admissions. Verify current timeframes with AHCA.

Typical startup costs to budget

- The home (lease or purchase), accessibility modifications, furnishings, and life-safety systems.
- Rehabilitation and clinical staffing or contracts (therapies, nursing, behavioral).
- Staff training, Level 2 screening, CPR/first aid; insurance and business setup.
- The AHCA license fee (confirm current TLF fee); your Policy & Procedure Manual and Forms Packet.

Getting your first residents

- **Miami:** build referral relationships with the region's trauma centers and rehabilitation programs (such as Jackson Memorial / Ryder Trauma Center and University of Miami rehabilitation), hospital discharge planners, BSCIP case managers, and the Brain Injury Association of Florida.
- **Tampa:** connect with Tampa Bay trauma centers and rehabilitation hospitals (such as Tampa General and area neuro-rehab programs), discharge planners, the VA, and the same statewide BSCIP referral channel.

In both markets, hospital and trauma-center discharge planners and BSCIP case managers are your primary referral sources, alongside families seeking specialized brain-injury rehabilitation.

Your Compliance Foundation

Opening the TLF is the first milestone; staying compliant with Chapter 400 Part XI and Rule 59A-17 — and ready for accreditation — is the ongoing work. Two companion resources are built for that: a Policy & Procedure Manual covering the licensure standards and the accreditation framework, and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone AHCA and an accreditor expect.

Before you rely on any step in this guide

Verify current requirements, forms, fees, and rates with AHCA (Long Term Care Services Unit) and, for funding, with BSCIP and the resident's Medicaid plan or insurer. A TLF is transitional and rehabilitation-focused, and Florida has no single brain-injury Medicaid per-diem — confirm each resident's funding before admission.